

Motor Vehicle Accident - Female (Child)

Algorithm 1208 | Child (0-17 years) | Gender at birth: Female

Definition

- Post-motor-vehicle-accident triage entry point, decomposed from NHS.UK's published whiplash guidance plus general trauma screening.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant, anticoagulated, intoxicated, or medically vulnerable, and what happened (impact, restraint, ejection, rollover, pedestrian/cyclist, helmet)? Include child/vulnerable-person safeguarding concerns.
2. [DURATION] When did this happen?
3. [OPEN_TEXT] What symptoms are being noticed now?
4. [YES_NO] If the patient is under 18, can they speak privately now, is the accompanying caregiver considered safe, and is there an approved safe callback method if contact is lost?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Back Pain

Was there no motor vehicle crash, with back pain or stiffness now being the primary concern?

Back symptoms without a motor vehicle accident belong in the Back Pain pathway.

1. Disposition level 100

If the patient is under 18, is harm occurring now, is there immediate danger from an accompanying or suspected caregiver, could the child be returned now to an unsafe person or place, or did contact end while an immediate threat remained unresolved?

Immediate danger, ongoing harm, an imminent unsafe return, or loss of contact during an unresolved immediate threat requires emergency action. Qatar 999 is the verified emergency route. Exact child-protection reporting, consent, documentation, and information-sharing requirements are local approval required.

2. Disposition level 78

If the patient is under 18 and there is no confirmed immediate danger, are they unable to speak privately, is caregiver safety uncertain, or is there another safeguarding concern requiring trained assessment?

An inability to obtain a private history, uncertainty about caregiver safety, or another safeguarding concern needs prompt trained assessment without automatically treating it as a confirmed emergency. The exact Qatar safeguarding pathway requires local approval.

3. Disposition level 100

Is there any loss of consciousness, severe bleeding, chest pain, severe difficulty breathing, inability to move a limb, severe abdominal pain, or numbness/weakness/pins and needles on one or both sides of the body?

Universal post-trauma emergency screen added ahead of any specific injury guidance - a motor vehicle accident can cause many types of serious injury simultaneously, so this general screen comes first before considering neck-specific (whiplash) or other single-body-part guidance.

3. Disposition level 70

Is there severe neck or back pain despite pain relief, tingling or pins and needles on one or both sides, problems walking or sitting upright, or a sudden electric-shock feeling in the neck, back, arms, or legs?

NHS.UK whiplash guidance lists these as reasons for an urgent primary-care review or urgent clinical review call - possible nerve involvement.

3. Disposition level 15

Is this mild soreness or stiffness (typical whiplash) with none of the features above?

NHS.UK guidance: continue everyday activities as able and use paracetamol/ibuprofen - resting the neck for long periods does not help recovery.

Advice

Immediate child safeguarding danger

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: harm occurring now or immediate danger; imminent return to an unsafe person or place; contact lost while an immediate threat remains unresolved.

Urgent private child safeguarding assessment

- Keep questions neutral and seek a private conversation only when it is safe. Do not disclose sensitive information to a caregiver whose safety is uncertain, do not confront or alert a suspected unsafe person, and do not promise confidentiality. Arrange prompt in-person assessment through the governance-approved Qatar

child safeguarding pathway and record only an approved safe callback method. If immediate danger emerges or contact is lost during an unresolved immediate threat, use the emergency branch and call Qatar 999. Exact reporting, consent, documentation, and information-sharing rules are local approval required.

- Call Back If: private history cannot be obtained safely; caregiver safety remains uncertain; new immediate danger or loss of contact.

Qatar emergency collision response

- Call Qatar emergency services on 999 and do not drive. Keep the person still in the position found unless there is immediate danger, breathing must be supported, or 999 directs movement. Do not remove a helmet, twist the spine, or make them stand/walk. Control severe external bleeding with direct pressure without removing embedded objects. If unresponsive and not breathing normally, follow 999 CPR instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening symptoms; loss of consciousness or abnormal breathing; weakness, numbness, severe bleeding, chest/abdominal pain, or pregnancy concern.

Urgent post-accident review

- Arrange same-day medical review for possible nerve involvement.
- Call Back If: numbness spreads; weakness develops.

Governed care after a minor collision

- Use gentle activity only as tolerated and avoid driving, hazardous work, sport, or forced neck movement while symptomatic. Do not use a collar unless prescribed. Medicine choice/dose requires age, weight, pregnancy, allergy, kidney/liver disease, ulcer/bleeding risk, anticoagulants, and current medicines under the approved Qatar pathway.
- Call Back If: pain worsens; numbness, weakness, confusion, vomiting, chest/abdominal pain, or breathing change.

First aid

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
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References

- NHS.UK, "Whiplash", <https://www.nhs.uk/conditions/whiplash/> (page last reviewed 03 January 2023)

Provenance

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